Office of the Deputy Secretary
Department of Health and Human Services

Re: HHS Health Sector AI RFI

February XX, 2026

Introduction

Parkland Health ("Parkland") appreciates the opportunity to submit comments in response to the HHS Health Sector Request for Information on accelerating the adoption and use of artificial intelligence in clinical care. These comments reflect Parkland's experience deploying AI within existing healthcare governance frameworks and provide concrete, experience-based feedback on regulatory and policy considerations affecting responsible AI adoption.

As one of the largest safety-net health systems in Texas and nationwide, the Dallas County Hospital District dba Parkland Health has deployed AI to enhance care delivery and operational efficiency since 2012. Today, Parkland operates dozens of production AI solutions ranging from machine learning risk scores supporting Level1 trauma care to generative-AI tools that assist with clinical documentation. As the public hospital for Dallas County, Parkland serves a patient population that is predominantly uninsured or covered by Medicaid, and responsibly deployed AI helps expand access to care for patients in underserved communities.

Core Themes

Based on Parkland's experience deploying AI within existing healthcare governance frameworks, the following themes reflect the operational considerations we believe are most relevant to HHS as it evaluates barriers to responsible AI adoption in clinical care.

Regulatory Consistency

Consistent, universal federal oversight of AI is a strategic and sustainable alternative to a by-state patchwork of regulations resulting in barriers for patients, healthcare providers, and solution developers alike. A national framework can help ensure consistent safety, privacy, and performance expectations while reducing unnecessary compliance complexity.

Public-Sector Parity

Equitable treatment of public- and private-sector providers is essential. Public-sector providers include large, complex health systems that support teaching, research, and the deployment of AI across a wide range of clinical settings. Consistent regulatory approaches across healthcare environments help avoid a two-tier system that could otherwise amplify disparities in quality, access, cost, innovation, patient experience, or population health.

Governance and Accountability

Health care providers already operate under one of the most comprehensive regulatory environments in the U.S., with extensive oversight governing patient safety, data privacy, clinical quality, and technology use. Layering additional, AI-specific requirements on providers risks duplicating existing safeguards and diverting resources from patient care, **while many AI-specific risks may be more effectively addressed through obligations placed on those who design, develop, and maintain AI systems.**

Illustrative Use Cases

The examples below illustrate how AI is currently being deployed within Parkland's existing clinical, safety, and governance frameworks, and the types of operational contexts that shape responsible use in real-world healthcare settings.

Workforce Safety
Parkland's Center for Clinical Innovation (PCCI) developed a predictive machine-learning model designed to help protect the Parkland workforce by identifying patients at higher risk of escalating to violent behavior early in the care journey. The model supports targeted use of clinical risk assessments and timely behavioral interventions, allowing care teams to focus attention where it is most needed. By shifting from reactive response to early risk identification, Parkland strengthens staff safety and situational awareness while maintaining compassionate, patient-centered care.

Early Warning System
In the inpatient setting, Parkland uses an Early Warning System (EWS), monitored by the Rapid Assessment Team, to proactively identify signs of patient clinical deterioration before they become life-threatening. This system analyzes real-time clinical data within the electronic health record and generates alerts that prompt timely bedside assessment and escalation when appropriate. By supporting early recognition and rapid response, EWS strengthens patient safety and accelerates intervention for Parkland's most vulnerable patients.

Ambient Listening
In the outpatient setting, Parkland uses ambient listening tools that allow physicians to engage patients face-to-face while large language models and generative AI assists with clinical documentation in the background. With patient consent, clinicians spend less time documenting and more time listening, observing, and connecting with patients. This approach reduces cognitive and administrative burden, enhancing both care quality and patient experience while preserving clinician oversight and accountability for the final note.

Translation Services
One area where regulatory requirements may limit the practical use of AI in clinical care is patient translation services for individuals with limited English proficiency (LEP). Section 1557 of the Affordable Care Act requires meaningful access to language services and, in certain circumstances, mandates qualified human review of machine-translated content. While these protections are critical to patient safety and equity, workforce shortages and cost constraints can limit the timely provision of written materials—such as discharge instructions, consent forms, or patient education—in patients' preferred languages, contributing to gaps in comprehension and outcomes. Parkland's experience suggests that advances in AI-enabled translation could help expand timely access to written materials for LEP patients.

What HHS Can Do

Based on Parkland's experience deploying AI within existing clinical, safety, and governance frameworks, HHS can play an important role in accelerating responsible adoption by providing clarity, flexibility, and alignment across federal policies—without introducing duplicative or prescriptive requirements. This is particularly important for safety-net health systems whose patients are disproportionately uninsured or covered by Medicaid.

1. **Support a coordinated federal approach to healthcare AI oversight.** HHS can align AI-related guidance and expectations across CMS, OCR, FDA, ONC, and other relevant agencies to reduce duplicative requirements and improve clarity for healthcare providers and vendors while preserving patient safety, privacy, and accountability.

2. **Reinforce existing provider accountability frameworks rather than adding duplicative AI mandates.** HHS can recognize that healthcare organizations already operate under robust clinical, safety, privacy, and liability regimes, and can emphasize organizational accountability for outcomes rather than prescribing uniform, technology-specific controls or workflow mandates.

3. **Clarify that providers may determine appropriate levels of human oversight based on risk and use case.** HHS can affirm that healthcare organizations are well positioned to assess when human-in-the-loop, human-on-the-loop, or human-out-of-the-loop AI is appropriate, consistent with clinical risk, patient impact, and existing standards of care—without default requirements for continuous human review.

4. **Consider modernizing interpretation of Section 1557 language access requirements for written materials (with safeguards).** HHS could consider clarifying that advanced AI-enabled translation may be used for written patient materials without mandatory qualified human post-editing when appropriate safeguards are met, including: (a) timely verbal explanation from a qualified interpreter or qualified bilingual staff member, (b) clear notice in the individual's preferred language that the document was machine-translated and that assistance or clarification is available upon request, and (c) continued provider responsibility for meaningful access, accuracy, and patient understanding.
5. **Encourage vendor transparency and performance substantiation to support responsible adoption.** HHS can develop regulatory guidance that requires AI developers and vendors provide standardized information regarding model performance, limitations, appropriate use cases, and monitoring requirements to enable responsible diligence and adoption decisions by healthcare organizations.
6. **Focus federal funding and pilots on healthcare deployers (with guardrails).** HHS can prioritize grants, demonstrations, or payment incentives that support healthcare organizations implementing AI in real-world clinical settings, including safety-net providers, while limiting exposure to fraud, waste, and abuse through deployer-focused design.
7. **Clarify that federal policy does not impede responsible automation of documentation and workflow.** HHS can clarify that appropriately governed AI tools supporting clinical documentation, care coordination, and administrative workflows are consistent with existing health IT, billing, and information-sharing rules, reducing ambiguity that contributes to clinician burden and access constraints.
8. **Adopt principles-based, future-ready policy that scales across the healthcare landscape.** HHS can favor flexible, outcomes-focused approaches that can evolve as AI capabilities advance so that smaller, rural, and safety-net providers—and the patients they serve—are not disadvantaged by overly prescriptive or resource-intensive mandates.

Conclusion

In closing, Parkland Health's decadelong experience deploying artificial intelligence demonstrates that, when implemented responsibly within existing clinical, safety, and accountability frameworks, AI can support improved patient outcomes, expand access to care, protect the workforce, and reduce administrative burden. We value HHS's efforts to solicit experience-based input through this RFI and believe that thoughtful, evidence-based approaches promoting consistency, equity, and responsible innovation can strengthen clinical care while maintaining patient trust and accountability. Thank you for considering Parkland's perspective as part of this process.

Respectfully submitted,

Joseph Longo, MBA, CDH-E, CHCIO
Senior Vice President/Chief Digital & Information Officer

Brett Moran, MD
Senior Vice President/Chief Health Officer